Tonsillitis clinical case presentation

Mr.Shravan Kashayap

Demographic Data

- Name: Abc
- Sex: female
- Age: 8 years
- Student
- Address Hoskote

Date of admission and examination 6 july 2020

Chief complaints

- Fever since 4 days
- Throat pain since 3 days
- Difficulty and pain While swallowing since 3 days

History of presenting illness

- Patient was apparently normal 4 days ago when she developed fever which was insidious in onset, progressive , continuous in nature, with no aggravating or relieved on taking medication. The fever was associated with malaise and lethargy.
- The throat pain was insidious in onset , gradually progressive , intermittent in nature and was aggravated by eating cold food or drinks, it was associated with difficulty in swallowing , pain during swallowing
- Patient gives history of similar recurrent episodes of about 2 episodes in this year and 3 episodes the previous year. With the last episode being 3 months back
- The patient change in the voice.
- The patient's mother on further questioning gave history of snoring and certain amount of day time sleepiness in the patient.
- There is no history of headache, cough , nasal discharge , epistaxis ,ear discharge , ear pain ,decreased hearing, ringing sensation in the ear.

Past history

- For the last episode she was under antibiotics and analgesics following which she recovered completely
- No history of bronchial asthma , diabetes , allergy to any drugs

Personal history

- Patient follows a mixed diet and her appetite is good ,
- Patient complains of disturbed sleep for the past 4 months
- bowel and bladder are regular ,

Family history

- No similar complaints in the family particularly siblings
- No history of any other systemic diseases in the family

summary

A child of 8 years was brought to the OPD by her mother with the child having complaints of fever since 4 days , throat pain since 3 days aggravated by cold food and associated with difficulty and painful swallowing since 3 days .

The patient gave positive past history of having similar recurrent episodes in the past year and 2 episodes this year .

The patient also on further questioning gave history of disturbed sleep, daytime sleepiness and snoring since 4 months.

Probable diagnosis- Acute exacerbation of chronic adenotonsillitis

Differential diagnosis – Acute pharyngitis

General physical examination

a child aged about 8 years , moderately built and nourished , was conscious and cooperative was well oriented to time place and person but breathing via open mouth.

Pallor – absent

Icterus – absent

Cyanosis – absent

Clubbing – absent

Oedema – absent

Generalised lymphadenopathy - absent

Vitals

- Temp – 100 deg farenheight
- Pulse – 80 bpm
- BP – 100/ 70 mmHg
- Respiratory rate – 16 breaths per minute

Systemic examination

- CVS S1 S2 heard , no murmurs
- RS bilateral air entry , Normal vesicular breath sounds heard
- CNS No focal neurological deficit higher mental function, motor function. And cranial nerve examination are all normal
- PA : soft and Non tender , no organomegaly

Oral cavity

- Mouth opening – Adequate
- Lip and oral commissures – normal
- Tongue - surface of the tongue was coated , no fissures or ulcers normal in size
- Floor of the mouth – normal , no ulcers or swellings, opening of submandibular duct normal
- Buccal Mucosa – normal in colour and surface
- Gingivo Buccal sulcus – Normal
- Dentition – Normal
- Retromolar trigone – Normal, opening of the parotid Duct normal.
- Hard Palate – Normal

Oropharynx

- Halitosis was present
- Uvula – Red and Congested
- Soft Palate – Red and congested
- B/L Ant Pillars – Congested
- Tonsils – Bilaterally enlarged (grade 3)
- B/L Post Pillars – Not seen
- Posterior Pharyngeal wall – Congested
- Indirect laryngoscopy – Not done

Local Examination of the ear

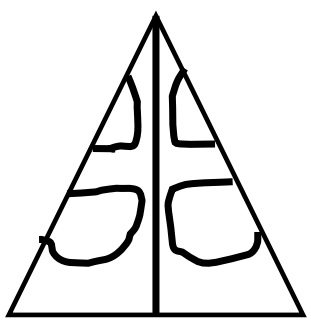
- Preauricular region- rt & lt – Normal
- Pinna- RT & Lt normal
- Post auricular region – Rt& Lt Normal
- Tragal sign negative on both sides
- EAC – both with and without speculum was normal on both sides
- Tympanic membrane both right and left – pearly white in colour , cone of light is seen clearly in the anteroinferior quadrant, handle of maleus is seen,
- Middle ear – not visible on both sides
- Facial nerve normal
- Eustachian Tube – Valsalva Manoeuvre – bulging of tympanic membrane seen on both sides

Functional Examination of the ear

- Rinnes test positive on both sides
- Webers test centralised
- Absolute bone conduction – same as examiner on both sides
- Fustula sign negative On both sides
- Spontaneous nystagmus absent
- Rohmberg test negative

Nose local examination

- External nasal frame work – normal on inspection , No tenderness on palpation.
- Tip raising test – ALA: normal
- Columella: Normal
- Vestibule: Normal
- Anterior Rhinoscopy
- Mucosa – Normal
- Septum – Normal and present in the midline
- Turbinates - Normal



Functional Examination of the nose

- Cold Spatula Test- Fogging/misting reduced bilaterally
- Cotton wool Test- Movement of cotton wisp on both sides was equal but reduced
- Posterior Rhinoscopy – Not done
- Test For olfaction- Not Done
- Cottles Test – Negative on both sides

Examination of Paranasal Sinuses

- Sinus Tenderness
- Maxillary sinus – Absent
- Frontal Sinus – Absent
- Ethmoidal Sinus - Absent

Examination of Neck

- Laryngeal Crepitus – Positive
- Bilateral jugolodigastric Lymphadenopathy is present and the nodes are tender , soft to firm in consistency , discrete , mobile in both horizontal and vertical planes and measure about 2x1 cm
- No scars, Dilated veins Or visible pulsations are present.

Provisional Diagnosis

- Acute exacerbation of chronic adenotonsillitis